



FAMILY PROSPECTUS

When your family needs *more than hope.*

A 24-bed residential drug rehabilitation centre in Lekki, Lagos. Twelve weeks of clinical, faith-integrated recovery — for families ready to begin.

Status

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Contents

A complete walkthrough for prospective families: who House of Refuge serves, the programme, what it costs, and exactly how every step is protected.

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01

SECTION ONE

When your family needs *more than hope.*

A clinical sanctuary, rooted in faith — for the family making one of the hardest decisions of their lives.



A clinical sanctuary, rooted in faith.

House of Refuge is a 24-bed residential drug rehabilitation centre in Lekki, Lagos — where international clinical standards and faith-integrated recovery are not in tension, but in partnership. Twelve weeks. A clinically-led programme. A son or daughter who comes home whole.

Every resident receives the same clinical standard.

The same psychiatrist, the same protocols, the same nursing care. What you pay for is privacy and amenity — never better medicine.

What you can expect, immediately

- An on-site consultant psychiatrist, five days a week, with 24/7 nursing and supervised detox throughout.
- Clinical care delivered to WHO and UNODC protocols, with daily multidisciplinary case reviews.
- A complete admission decision in writing — accept, decline, or referral — within five working days of clinical assessment.
- A mandatory pastoral interview with Pastor Tony Rapu, founder of Freedom Foundation, attended by patient and family.
- An honest answer about your loved one's fit before any commitment beyond the deposit.

Licensed and accountable to

Lagos State Ministry of Health · National Drug Law Enforcement Agency (NDLEA) · clinically governed by an independent advisory board · operated by the Freedom Foundation NGO · managed day-to-day by ConsultForAfrica.

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SECTION TWO

Is House of Refuge right for your loved one?

Recovery is not something that can be done *to* a person — it is something we walk through *with* them. We are honest about who we can and cannot help.



The Willingness Gateway.

The willingness of the person seeking treatment is the single most important factor in long-term recovery. It is the very first thing we assess — before any clinical workup, before any commitment beyond a free conversation.

NON-NEGOTIABLE

Voluntary willingness is the gateway to admission.

Before any clinical assessment, our team holds a structured conversation with your loved one — assessing readiness for change using the URICA framework and motivational interviewing.

If they are not yet willing, we do not force admission. Instead, we offer an Outpatient Engagement Pathway to meet them where they are.

What this means in practice

- **We never simply turn a family away.** Even if residential treatment is not the right fit today, we offer a pathway, written referrals, and continued contact.
- **We talk to your loved one directly.** The willingness conversation is with the person who would be admitted — not with their family on their behalf.
- **Your loved one can change their mind.** A "no" today can become a "yes" later.



Five inclusion criteria.

All five must be met for residential admission. The criteria exist to keep every resident safe — your loved one and the others sharing the residence.

1 Voluntary Willingness

Your loved one personally consents to treatment and demonstrates readiness for change.

2 SUD Diagnosis

A clinically confirmed substance use disorder, established through ASI or ASSIST screening.

3 Medical Stability

Cleared on the nine pre-admission medical tests and medically safe to undergo supervised detox.

4 Functional Capacity

Able to participate in daily therapy, group sessions, and structured residential life.

5 No Immediate Risk

No active threat of harm to self or others requiring acute psychiatric containment.

The nine pre-admission medical tests

HIV · Hepatitis A & B · Urine Drug Screen · Full Blood Count · Liver Function Tests · Malaria · Widal · Chest X-Ray · Blood Glucose. All conducted under our clinical supervision; results stay on the medical file throughout the programme.



What we cannot safely treat in residence.

We are honest with families about our limits. The presentations below require an acute psychiatric or specialist setting — not a residential rehabilitation programme. If your loved one falls into one of these categories, we will help you find appropriate care.

Active psychosis

Severe suicidal or homicidal risk

Drug-induced psychosis

Active antipsychotic medication need

Severe cognitive impairment

Active legal detention

Dependent children requiring co-habitation

How we handle a "no"

- The clinical decision is delivered to the family **in writing**, with the reasoning behind it.
- Every declined applicant receives a **written referral** to a more appropriate care setting.
- The 600,000 portion of the deposit is **refunded in full** within 14 days. The 400,000 assessment fee is retained — the clinical work was done.

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SECTION THREE

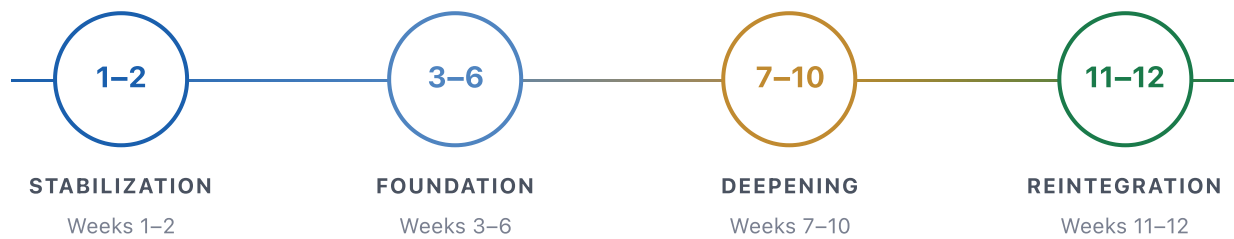
Twelve weeks.
Four phases.
One clinically-led pathway.

Each phase has defined goals, weekly clinical reviews, and clear family touchpoints. You will never wonder where your loved one is in their recovery, or what comes next.



The twelve weeks at a glance.

The HOR programme is delivered across four clinical phases over twelve weeks. The phases progress with your loved one — each one builds on the last.



What you can expect each week

- **Daily group therapy** — CBT, 12-step, and faith-based work, delivered by qualified clinicians.
- **Weekly individual counselling** — daily on the Signature tier, three times weekly on Premium.
- **Weekly multidisciplinary case review** — your loved one's progress is examined every week.
- **Faith and chaplaincy programme** — daily, woven into the daily schedule.
- **Family touchpoints** — at least one every fortnight, more often by clinical request.

Where clinically indicated, the programme is extendable to six months at the same monthly tier rate. Extension is recommended by the multidisciplinary team — never sold.



Phases 1 & 2 — Stabilization and Foundation.

The first six weeks: settling the body, stabilising the mind, and laying the therapeutic foundation that the rest of recovery will build on.

PHASE ONE

Medical Stabilization

Weeks 1–2

- Supervised medical detox with 24/7 nursing
- Vitals, nutrition, and sleep stabilization
- Full programme orientation
- Initial spiritual care + chaplaincy

FAMILY TOUCHPOINT

Welcome call within 72 hours. Orientation pack delivered. Visitation begins from Week 2.

PHASE TWO

Therapeutic Foundation

Weeks 3–6

- Group CBT and relapse-prevention work
- Weekly 1:1 counselling (daily on Signature)
- Psychoeducation, life skills, fitness
- Week 4 multidisciplinary treatment review

FAMILY TOUCHPOINT

First family therapy session by Week 4. Sunday visitation each week. Treatment review report shared.



Phases 3 & 4 — Deepening and Reintegration.

The second six weeks: trauma processing, family-systems work, and a structured exit into a real life ready to receive your loved one back.

PHASE THREE

Deepening & Skills

Weeks 7–10

- Trauma processing and family-systems work
- Vocational assessment and planning
- Personal Relapse Prevention Plan (PRPP)
- Week 8 progress review with the MDT

FAMILY TOUCHPOINT

Family-systems sessions deepen. 24-hour and 48-hour passes become available based on conduct.

PHASE FOUR

Reintegration

Weeks 11–12

- Pre-discharge clinical assessment
- Aftercare and reintegration plan
- Graduation ceremony
- Alumni Programme enrolment

FAMILY TOUCHPOINT

Pre-discharge family meeting. Graduation ceremony. Year-one aftercare schedule confirmed.

The graduation criteria, in brief

Six criteria must all be met for planned discharge: clinical stability, behavioural compliance, psychological readiness (with insight and a written PRPP), relapse-prevention competence, spiritual formation, and reintegration readiness.

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SECTION FOUR

Programme fees, admission flow, and exactly how your money is held.

A clinical decision and a financial decision happen together. Below is exactly how both work — what you pay, when, what each part of the deposit covers, and what protects you at every step.



Two paying tiers. One clinical standard.

Same psychiatrist, same protocols, same nursing, same WHO/UNODC framework. What you pay for is privacy, amenity, and the rhythm of one-to-one contact.

SIGNATURE

3 beds · always limited

NGN 4,500,000 /month

3-month total: NGN 13,500,000

- ✓ Private en-suite room — single occupancy
- ✓ Daily 1:1 therapy with a clinical psychologist
- ✓ 4 family sessions per month
- ✓ Personal chaplain — weekly
- ✓ Concierge admissions with private transport
- ✓ Signature menu — broader, with premium options
- ✓ Vocational planning + aftercare

PREMIUM

19 beds

NGN 2,800,000 /month

3-month total: NGN 8,400,000

- ✓ En-suite room — 2-bed shared
- ✓ 1:1 counselling — 3 sessions per week
- ✓ 2 family sessions per month
- ✓ Group chaplaincy — daily
- ✓ Concierge admissions + dedicated keyworker
- ✓ Premium menu (Signature menu add-on available)
- ✓ Reintegration plan + aftercare

A NOTE ON FINANCIAL ASSISTANCE

Need-based admission is available to families experiencing genuine financial hardship. Applications are reviewed independently by the **Freedom Foundation Financial Assistance Committee**. The clinical standard does not change for need-based residents. Placements operate on a **separate waitlist with unpredictable wait times** — this tier is fully donor-funded, so capacity is extremely limited and dependent on funding availability. Apply at houseofrefugeng.org/assistance. Sponsorship inquiries: **0911 277 7600**.



The admission flow — six stages.

Six stages between your first call and your loved one's first day. Each is documented; nothing happens behind closed doors.

1

Self-assessment

Free · About 30 minutes online

Structured intake form: substance history, medical history, family context, willingness for treatment. Becomes the working file for the clinical team.

2

Deposit

NGN 1,000,000 · After self-assessment review

Once the file passes initial review, you are invited to pay the 1M deposit. Split: 400k assessment fee + 600k held toward Month 1.

3

Clinical assessment

Within 7–14 days of deposit

Consultant psychiatrist + 9 pre-admission lab tests + MDT case review. Decision in writing within 5 working days.

4

Pastoral interview

Pastor Tony Rapu · Patient and family · Booked by admissions

A **mandatory** discernment conversation with Pastor Tony Rapu, founder of Freedom Foundation. Patient and family attend together. Completes the admission gate alongside the clinical decision.

5

Active waitlist (if needed)

Up to 6 months · Monthly check-in · Renewable

Beds are limited (24 in total). Your 600k is held — reserving your place. Admissions checks in monthly. Withdraw any time for full 600k refund.

6

Admission

Bed offered, family confirms, admission scheduled

600k applied to Month 1. Balance settled before admission day (3.9M Signature, 2.2M Premium). Phase One — Medical Stabilization — begins.



The 1M deposit, plain.

The admission deposit is NGN 1,000,000. It is split into two parts. We tell you exactly where every Naira goes — published, not in the small print.

DEPOSIT BREAKDOWN · ITEMISED

Where every Naira goes.

NGN 400,000

Clinical assessment fee

- Psychiatrist consultation (structured interview)
- 9 pre-admission lab tests (HIV, Hep A&B, UDS, FBC, LFTs, Malaria, Widal, CXR, Blood glucose)
- MDT case review + written decision
- Referral to appropriate setting if declined

Consumed once the psychiatrist begins your file.

NGN 600,000

Held toward Month 1

- Reserves your place on the active waitlist
- Applied directly to Month 1 fees on admission
- Refundable in full if you withdraw or we decline
- Refundable if no bed opens within the hold period

Held — never consumed unless you admit.

A note on prior fees: the previously published 150,000 clinical-admission-assessment fee and the 50,000 processing fee have been **abolished** and folded into the 400,000. Families now see one number and one breakdown.



Refund policy in plain language.

We refund every Naira we have not earned. The full table is below — published, not in the small print.

If...	Then...
Within 48 hours of paying the deposit, you change your mind	Full NGN 1,000,000 refunded
You withdraw between 48 hours and the psychiatrist consult	600k refunded ; 400k retained for clinical work
Our psychiatrist clinically declines admission	600k refunded + written referral; 400k retained
You withdraw more than 14 days before admission day	600k refunded
You withdraw within 14 days of admission day	50% of 600k refunded; 50% retained as bed-hold cancellation
We cannot offer a bed within your active waitlist hold	600k refunded ; 400k retained
You withdraw voluntarily during the waitlist hold	600k refunded in full, anytime
You decline a slot offer once it is made	600k refunded ; place released, no haggling
Within 72 hours of admission, your loved one withdraws	80% of total Month 1 paid; 400k not refundable
Withdrawal after 72 hours of admission	No refund (no exceptions)
Medical discharge after Month 1 (clinical reasons)	Months 2+ refunded pro-rata



How the waitlist actually works.

A 24-bed facility means there will be times when no bed is immediately available. The active waitlist exists to make the wait fair, transparent, and reversible at any moment.

- **Active hold lasts 6 months.** Renewable on confirmation if you wish to keep waiting.
- **Monthly check-in.** Admissions contacts you each month. Two missed contacts releases the hold and refunds 600k.
- **Withdraw whenever you need to.** By phone, email, or in person. 600k refunded in full.
- **One decline is a clean exit.** If a slot is offered and you decline it, that's your decision. We refund the 600k. No haggling.
- **Re-assessment after 6 months.** If you wait beyond six months, a brief re-assessment (NGN 100,000) updates the clinical file before admission.

Why we hold the deposit through the wait

Holding the 600k while a family waits is not a mechanism to keep their money — it is the mechanism by which they keep their place in the queue. Releasing the deposit means releasing the slot. We make this trade-off plainly because ambiguity at this stage is the cause of more lost trust than the wait itself.

05

SECTION FIVE

The rhythm of recovery.

Plate by plate, hour by hour. Recovery is not a single decision — it is a daily one, repeated for ninety days.



Meals as part of the medicine.

Detox is exhausting work for the body. Sleep, hydration, blood sugar, and basic nutrition all suffer through addiction — and recovery is partly the slow business of putting them back together.

Our catering partner **CookedIndoors** prepares every meal fresh, with menus shaped alongside the clinical team. The food is warm, varied, and deliberately familiar — meals residents recognise from home, not the bland institutional cooking that defines too many rehabilitation centres. Snacks are kept light and designed to be shared: a small daily ritual of community, taken at the longer gaps between sessions.

NUTRITION MEETS CLINICAL CARE

Every resident's plan is reviewed by the clinical team at intake — and adjusted through the programme as recovery progresses. **Halal, vegetarian, diabetic, hypertensive, allergy-led** — every dietary need is accommodated, without families having to negotiate it.

Why we lean into meals so heavily

- Stable blood sugar reduces relapse cravings during early recovery.
- Restored sleep rhythm depends on regular evening meals.
- Communal dining is itself therapeutic; it teaches the rhythms of life outside the residence.



The daily rhythm.

Meals slot quietly into the wider programme schedule — never disruptive, always anticipated.

8:00 AM	Breakfast, after morning prayer
10:30 AM	Mid-morning snack — shared
12:30 PM	Lunch
3:30 PM	Afternoon snack — shared
6:00 PM	Dinner, before evening chapel

A note on tiers

Signature residents eat off the **Signature menu** — broader, with additional protein options and weekend specials. **Premium** residents eat off the **Premium menu**, with the option to upgrade to full Signature menu access for any month at NGN 30,000. The clinical team reviews every plate; every dietary need is met regardless of tier.

The medicine doesn't change.

Same psychiatrist. Same protocols. Same nursing care. The kitchen handles privacy and amenity — never better medicine.



WHEN YOU ARE READY

Begin admission.

An admissions counsellor will respond within one working day, in confidence. There is no obligation, and no commitment beyond the conversation, until the self-assessment is reviewed and you choose to continue.

PHONE

0911 277 7600

Admissions · 9 AM – 6 PM, Mon–Sat

ONLINE

houseofrefugeng.org

Self-assessment form · ~30 minutes

Location

Lekki, Lagos ·
Nigeria

Capacity

24 residential
beds

Programme

12 weeks · Extendable to 6
months

Confidential

A Freedom Foundation
Initiative